WHY IT IS RISING AGAIN



ASHIQUE MUHAMMED K

Tuberculosis itself is hardly new in India: it has the world's earliest written record of the disease, from circa 1500 BC. The country's National Tuberculosis Elimination Programme to eradicate this communicable disease has been running for over 50 years now. Once known as a poor man's disease, TB no longer discriminates on the basis of class. Nor is it restricted to the lungs; the bacteria can just as easily settle in the kidneys, brain or spine. Treatment, typically, is in two phases. In the initial phase, a set of four first-line drugs—isoniazid, rifampicin, pyrazinamide and ethambutol—are prescribed for two months. Then comes the continuation phase, in which isoniazid, rifampicin and ethambutol continue for another four months.

Years of antibiotic use and misuse have made the bacteria adept at evading these drugs. According to the National Anti-TB Drug Resistant Survey (DRS) from 2014-16, a quarter of the TB patients in India were resistant to one or the other of these drugs, and 1.3 per cent of these were XDR-TB cases. According to the India TB Report 2023, the number of MDR-TB cases diagnosed in 2022 was 63,801. More specifically, MDR-TB is resistant to isoniazid and rifampicin and XDR-TB is resistant to rifampicin, any fluoroquinolone, and at least one of the three injectible second-line drugs—amikacin, kanamycin or capreomycin. In recent years, DR-TB has begun to be treated with two oral drugs—bedaquiline and delamanid—which are available at government hospitals or at designated private hospitals. These drugs are more effective and have fewer side-effects than the older ones used for DR-TB.

If human ignorance is a factor in the birth of resistant strains, it also plays more than a walk-on part in the spread of this communicable disease—and the toll it continues to exact. Since its only symptom initially is a prolonged cough, many do not know they have TB until it is too late. "If someone has a cough for more than two to three weeks and other problems have been ruled out, then one must look for the possibility of TB," says Dr Sandeep Nayar, head of the chest and respiratory diseases department at the BLK Max Super Specialty Hospital in Delhi. Delay can not only worsen an individual's case, but can also infect others in that time. Pulmonary TB is highly infectious. People with active TB can infect five to 15 people through close contact over the course of a year. The disease spreads through the air when an infected person coughs or speaks. When another person breathes in the bacteria, it settles in the lungs, where it can lie dormant, a stage called latent

▲

DEBSHREE LOKHANDE | 35, Architect, Pune

A LONG, PAINFUL BATTLE

Debshree's life changed forever when she was diagnosed with XDR-TB in 2014. By then, three years of incorrect reports and medical advice had already worsened her condition. The tests showed that she was resistant to three of the four drugs in the standard four-drug kit for TB. It was only when Debshree was prescribed bedaquiline on compassionate grounds that she was able to recover. But her treatment was excruciatingly painful. A dreaded side effect was loss of speech and hearing. "This made me feel even more isolated. My sister was the only one who managed to communicate with me, using sign language," she recalls. What's worse, doctors didn't advise her about the infectiousness of TB, and Lokhande's sister also ended up contracting the disease. Today, though Debshree has been cured completely, there are still some people who avoid meeting her. "I am a committed and outspoken advocate who is sharing her story, so that people are more aware of the realities of the disease," she says.